

MR BRAIN W AND WO IV CONTRAST

Results

Study Result

Narrative & Impression

UNIVERSITY HEALTH

4502 MEDICAL DRIVE, SAN ANTONIO, TX 78229

PH: 210-358-8532

PROCEDURE(S): MR BRAIN W AND WO IV CONTRAST

ACCESSION(S): UH5919153

DATE OF EXAM: 4/10/2026 3:38 PM

REFERRING PHYSICIAN: SHRADDHA M DALWADI

EXAM: MRI brain with and without IV contrast.

TECHNIQUE: Multiplanar, multisequence MRI of the brain was performed both before and after the administration of intravenous gadolinium contrast.

CONTRAST: 10 mL Gadavist

HISTORY: Malignant neoplasm of unspecified part of unspecified bronchus or lung (CMS/HCC) [C34.90]

Malignant neoplasm of unspecified part of unspecified bronchus or lung (CMS/HCC) [C34.90]

COMPARISONS: MR brain perfusion with and without contrast 1/24/2026

FINDINGS:

Brain parenchyma/contrast enhancement: No acute infarction or acute intraparenchymal hemorrhage. Periventricular and deep subcortical white matter T2/FLAIR hyperintensities consistent with mild chronic microvascular ischemic changes. No new definite enhancing lesion is identified.

Heterogenously enhancing lesions as follows:

- * Midline cerebellar vermis lesion is increased in size now measuring 2.2 x 2.6 x 2.2 cm (previously measuring 1.4 x 2.4 x 1.3 cm). Mild increase in degree of adjacent perilesional edema. Interval increase in amount of intralesional susceptibility artifact.

- * Left lower cerebellar lesion is increased in size now measuring 1.1 x 1.0 x 1.1 cm (previously measuring up to 0.6 cm). Mild perilesional edema.

* Left frontal lobe lesion is significantly decreased in size now measuring up to 9 mm (previously measuring up to 1.5 cm. Significant interval decrease in amount of perilesional edema. This lesion demonstrates susceptibility artifact.

* 2-3 mm enhancing nodular lesions in the left lateral cerebellum and in the bilateral occipital lobes are mildly more pronounced from the prior exam when they were very subtle and small.

Contrast Enhancement: No abnormal enhancement to suggest tumor or infection.

Ventricular system: No hydrocephalus or midline shift.

Extra axial spaces: No epidural, subdural, or subarachnoid hemorrhage. No basal cistern effacement.

Vascular system: No loss of the major vascular flow voids.

Visualized paranasal sinuses: Mild bilateral maxillary sinus mucosal thickening. The bilateral mastoid air cell complexes are clear.

Visualized orbits: No ocular or retrobulbar pathology.

Calvarium and skull base: No osseous destruction.

Pituitary and Pineal Glands: No mass lesion.

Soft tissues: No significant abnormality.

IMPRESSION:

1. Mild heterogeneous nodular lesion involving the left paramedian lower vermis/medial cerebellar hemisphere has increased in size with increase in the surrounding edema.
2. Punctate areas of nodular enhancement in the left lateral and lower cerebellum and in the bilateral occipital lobes are mildly more pronounced from the prior exam. These findings are concerning for disease worsening.
3. The previously seen heterogeneously enhancing lesion in the left paramedian frontal lobe has decreased in size but decrease in surrounding edema and interval gradient susceptibilities which may suggest posttreatment changes.
4. The heterogeneous enhancing lesion involving the upper and mid cerebellar vermis, closely related to the posterior and lateral margins of the fourth ventricle, is mildly increased in size but the surrounding edema does not show prominent increase and interval gradient susceptibility is are seen. Findings may suggest treatment-related changes and needs clinical correlation.
5. Ventricles are grossly stable in size. No interval intracranial hemorrhage, prominent vasogenic edema, or acute infarction is otherwise seen in the rest of the brain.

I, Manoj Kumar, have personally interpreted the image(s) and dictated this examination or I have reviewed the image(s) as well as the residents interpretation and agree with the interpretation.

Final Signed by Manoj Kumar, on 4/12/2026 10:59 AM

Images

Images

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